

	EMERGENCY MEDICAL SERVICES AGENCY	POLICY NUMBER	760
	A Division of the Merced County Department of Public Health	Effective Date	11/2025
Authority: Health and Safety Code, Division 2.5, and California Code of Regulations, Title 22, Division 9, Chapter 4, 1797.220		Initial Date:	09/2018
		Review Date:	07/2028

AIRWAY MANAGEMENT	
ADULT	PEDIATRIC
BLS	
Assess Vitals / Obtain SpO ₂ / Oxygen via selected device based on the patient's condition, titrate SpO ₂ to > 94%. Airway Adjuncts – OPA/NPA as needed to control the airway. Begin CPR if indicated.	
Nasal Cannula: 2 – 6 LPM Non-rebreather mask: 10 – 15 LPM BVM ventilations: 10 breaths/min CPAP when indicated.	Nasal Cannula: 2 – 6 LPM Non-rebreather mask: 10 – 15 LPM BVM ventilations: 12 – 20 breaths/min CPAP when indicated (≥ 8 years of age).
<u>Supraglottic Airway Device (SAD)</u> EMT Optional Scope of Practice Only – Age 12 years of age or older	
Verify placement by ALL the following: • Rise and fall of the chest • Bilateral lung sounds • Negative epigastric sounds • Condensation in the tube • Application of Colorimetric CO₂ detector in 1 minute or less Reassessment: • All SAD patients must be continuously assessed • Any significant movement, emesis, or change in clinical condition should be reassessed using a Colorimetric CO₂ detector and physical examination • If at any time, clinical indicators suggest that the SAD is not in communication with the trachea, the airway must be immediately removed, and reinsertion of the SAD attempted	

Signatures on File

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EMS Administrator

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EMS Medical Director

ALS

Follow BLS procedures if applicable.

Supraglottic Airway Device (SAD)

Indicated Paramedic application for patients weighing 2 kg and above

Verify placement by ALL the following:

- Rise and fall of the chest
- Bilateral lung sounds
- Negative epigastric sounds
- Condensation in the tube
- Application of Waveform ETCO₂ detector in **1 minute or less**

Reassessment:

- All SAD patients must be continuously assessed using Waveform EtCO₂ Capnography
- Any significant movement, emesis, or change in clinical condition should be reassessed using Waveform EtCO₂ Capnography and physical examination
- If at any time, clinical indicators suggest that the SAD is not in communication with the trachea, the airway must be immediately removed, and reinsertion of the SAD attempted

Oral Endotracheal Intubation (ETT)

Adult Patients Only

Verify placement by ALL the following:

- Rise and fall of the chest
- Bilateral lung sounds
- Negative epigastric sounds
- Condensation in the tube
- Application of Waveform EtCO₂ in **1 minute or less**

Reassessment:

- All intubated patients must be continuously assessed using Waveform EtCO₂ Capnography
- Any significant movement, emesis, or change in clinical condition should be reassessed using Waveform EtCO₂ Capnography and physical examination
- If at any time, capnography indicates that the tube is not in communication with the trachea, the airway must be immediately removed, and reintubation attempted

ETT Policy

Do **not** make more than two (2) attempts total per patient to perform endotracheal intubation.

An attempt to intubate is defined as placement of the laryngoscope blade in a patient's mouth with the intent to intubate.

If either unsuccessful after 2 attempts, or intubation not felt possible, insert an SAD, or use BLS airway techniques.